AIRA
AMERICAN IMMUNIZATION REGISTRY ASSOCIATION

June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Dr. Thomas Keane
National Coordinator for Health Information Technology
Assistant Secretary for Technology and Policy, Office of the National Coordinator
Department of Health and Human Services,
Mary E. Switzer Building, Mail Stop: 7033A
330 C St. SW
Washington, DC 20201

RE: Request for Information; Health Technology Ecosystem

Dear Dr. Oz and Dr. Keane,

On behalf of the American Immunization Registry Association (AIRA) we are pleased to submit comments on the Request for Information recently released by the Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology and Policy, Office of the National Coordinator (ASTP/ONC). These comments are a compilation of the input of our members which include over 100 organizations representing Public Health Immunization Information Systems (IIS), IIS implementers and vendors, non-profit organizations and partners. IIS interface with a broad range of stakeholders, including providers, pharmacists, schools, child care facilities, health plans and payers, among others.

IIS and our partners are, quite obviously, very invested in promoting smooth interoperability to ensure broad data use. At the point of clinical care, an IIS provides consolidated immunization records and forecasts to support clinical

decisions. At the population level, an IIS provides aggregate data and information on vaccinations for surveillance, program operations and public health action. It is critical that the role of Public Health is recognized as a key part of health IT strategy moving forward.

To that end, we have specific input on key points posed in the RFI presented on the following pages. Please feel free to contact me with any questions: mbkurilo@immregistries.org.

We greatly appreciate the opportunity to respond to the many critical questions posed throughout this document, and we look forward to continuing to collaborate to ensure high-value health IT interoperability across the ecosystem.

Sincerely,

Mary Beth Kurilo, MPH, MSW
Senior Director of Health Informatics
American Immunization Registry Association (AIRA)

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General Comment (Regarding Public Health Representation)

AIRA would like to emphasize the critical importance of integrating public health perspectives into the ongoing discussions about the future of the health IT ecosystem. This Request for Information (RFI) underscores the need for a more interconnected and patient-centric health technology infrastructure.

Public health entities, including state and local health departments, play a pivotal role in disease prevention and control. Their involvement in health IT initiatives ensures that public health data is accurately captured, shared, and utilized to inform public health strategies and policies.

Excluding public health from these conversations risks overlooking the broader implications of health IT decisions on community health outcomes, and ultimately, on the health of every individual American. Incorporating public health expertise can help identify potential gaps in data interoperability, ensure that health IT systems support population health management, and facilitate timely responses to emerging infectious diseases.

We urge CMS and ASTP to actively engage public health stakeholders in the development and implementation of health IT policies and systems. By fostering collaboration between healthcare providers, technology developers, and public health entities, we can create a more robust health IT ecosystem that serves the needs of all Americans.

Specific Questions and Responses

PC-10.c What use cases could have a significant impact if implemented through TEFCA?

We recommend that the function of responding to FHIR queries from authenticated PH programs through FHIR APIs become a requirement of the Public Health Exchange Purpose for TEFCA. To date, we are learning of Public Health Agencies who have modernized their systems to perform FHIR queries only to be unable to receive responses as responding is not a requirement of TEFCA participants. Requiring response to a public health query will reduce provider burden through reduction in phone calls and other manual interventions.

New public health and individual access exchange purposes for immunizations would enable better directed exchanges with immunization information systems. For example, the current Individual Access Service XP (T-IAS) may be too broad for public health systems such as IIS. For persons looking for their immunization records from public health – a common need – it would be ideal to have a focused XP (e.g., T-IAS-IZ) for proper QHIN routing. IIS would likely be unnecessarily overburdened with broad T-IAS query volume. IIS

could also benefit from a level 2 Public Health Exchange Purpose similar to electronic lab reporting (ELR) and electronic case reporting (ECR). Without this, it's unclear how standardization of immunization reporting would be achieved with the same successes we see today found in (f)(1) EHR certification criterion.

PR-5.g Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints: g. Bulk FHIR—Do you support Group ID-based access filtering for population-specific queries?

This question is specific to providers implementation of FHIR APIs. However, this response is from the perspective of public health – and specifically Immunization Information Systems (IIS). Providers regularly query IIS for cohorts of patients. Unfortunately, this is currently being performed using the legacy HL7 V2 query intended for single patient access. Without implementation of a Bulk FHIR solution, providers are sequentially querying IIS for their entire cohort. This methodology is costly to IIS to maintain – both in regard to monthly cloud hosting costs and performance metrics. Bulk FHIR support by IIS is limited today and funding for IIS is needed to encourage all IIS to offer a Bulk FHIR solution so long as providers and payers are encouraged to use Bulk FHIR for large cohorts of patients.

PA-5.a How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

As stated in response to the question above (PR-5.g), without implementation of a Bulk FHIR solution, payers are sequentially querying IIS for their entire cohort of members. This methodology is costly to IIS to maintain – both in regard to monthly cloud hosting costs and performance metrics. We support and advocate for funding for IIS to encourage all IIS to offer a Bulk FHIR solution, with the assumption that providers and payers will be encouraged to use Bulk FHIR for large cohorts of patients.

TD-1 What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Immunization Information Systems (IIS) need direct support from CMS to implement the newest standards and technologies to interact with nationwide networks at scale. IIS maintain official immunization records for people of all ages and is a critical resource for patient care, particularly among older Americans where immunizations are especially critical. CMS could provide funding or technical assistance to help IIS upgrade their infrastructure and implement the latest standards and technologies, ensuring data interoperability with emerging digital health products.

TD-2.b What data sources are most valuable alongside the data available through the Blue Button 2.0 API?

The most valuable data sources for public health alongside the Blue Button 2.0 API include data from Electronic Health Records (EHRs), in alignment with the United States Core Data for Interoperability (USCDI). Access to EHR data allows public health programs to gather comprehensive, patient-level information, such as immunization records, diagnoses, lab results, and medication histories. The inclusion of USCDI data further enhances the interoperability of health data, enabling the standardized exchange of crucial health information across systems. This data enables public health programs to collect, analyze, and report on health trends, ensuring timely and accurate interventions. Integrating these data sources with Blue Button 2.0 enhances the ability to monitor population health, support disease surveillance, and improve care coordination across the healthcare system, while ensuring alignment with national data standards.

TD-2.c What obstacles prevent accessing these data sources today?

Although systems using Certified Health Information Technology have the technical capacity to make USCDI mandated data available, our experience is that holders of this data, including healthcare organizations, are often reluctant to engage with public health and ensure API-based access to clinical data in alignment with public health authority. All holders of patient data must see appropriate data queries by public health as a core interoperability responsibility. Trust must be built between public health programs seeking data for with they have clear authority and the holders of this data.

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?

Historically, the public health experience has been that patient identification and matching is a complex but vital piece of public health data management and utilization. We strongly recommend including the public health perspective in deliberations regarding the implementation of digital identity within the healthcare realm. Too often demographics are either incomplete or inconsistent for definitive patient matching without costly human intervention. Integrating the needs of public health with those of other sectors of healthcare and government will be critical to ensuring that jurisdictions and their trading partners implement digital identify functionality in a standards-based method, thus also enabling broader interoperability with those data.

Long term investment into digital identity should improve patient identification, patient matching within and across systems along with improving the accuracy and completeness of patient demographics.

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

a. Does it contain the full extent of data elements you need?

AIRA has regularly commented on USCDI versions. We have struggled to see evolution of the USCDI data elements specific to our comments but will continue to comment on them to encourage the adoption of important immunization focused data elements we believe remain missing. We have also recommended some reorganization of the Immunization terms we believe would add clarity to the Immunizations class of data elements. A first step should be to align USCDI with the Required and Must Support data elements found in the US Core Immunization Profile.

c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

The additions and reorganization suggestions from AIRA would be a value add. The elements being requested (e.g., date administered, admin/historical) have been exchanged for a decade plus outside of USCDI. The reorganization suggestions would add clarity to currently duplicate naming conventions.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The (f)(1) immunization certification criterion has been a resounding success. The last Immunization Information System Annual Report (2023 data – reported in 2024) showed 161,208 live HL7 2.5.1 connections. Our first-hand experience has been that the (f)(1) criteria have been a significant motivating force in the establishment of this impressive number of direct connections.

TD-9. Regarding certification of health IT:

d. What policy changes could CMS make so providers are motivated to respond to API based data requests with best possible coverage and quality of data?

Exploring FHIR subscription functionality could be a worthwhile effort. Subscriptions offer the opportunity to reduce the number of redundant queries for the same patient when nothing has changed. The FHIR standards for subscriptions and connectathons are progressing. Further investigation/piloting is needed to understand the burden it will (or won't) place on users who will be responsible managing subscriptions.

e. How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

The bulk "import" approach used by UDS+ could be a significant improvement for large scale reporting, legacy data migration, and many other bulk pushes of data. This should continue to be explored.

TD-15. Regarding bulk FHIR APIs:

a. How would increased use of bulk FHIR improve use cases and data flow?

As mentioned above, in the absence of implementation of a Bulk FHIR solution, providers and payers are sequentially querying IIS for their entire cohort of patients. This methodology is costly for IIS to maintain – both in regard to monthly cloud hosting costs and performance metrics. Bulk FHIR support by IIS is limited today and funding for IIS is needed to encourage all IIS to offer a Bulk FHIR solution so long as providers and payers are encouraged to use Bulk FHIR for large cohorts of patients.

b. What are the potential disadvantages of their use?

One challenging area – not necessarily a disadvantage – has been our experience as a participant in the Helios FHIR Accelerator Bulk Data Priority Area. This has highlighted the challenges of patient identification between systems in bulk. This is a prerequisite for Group ID-based access in the absence of a patient identifier known by both systems. Additional standards, testing, and implementation support are required before Group-based bulk data is likely to be commonplace. This complexity is not necessarily directly created by Bulk FHIR. It exists in single query transactions as well but is less noticeable given the synchronicity of single query transactions compared to the volume of records exchanged in asynchronous Bulk (and the subsequent handling of bulk non-matches, mismatches, etc.).

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

Public Health has become a significant part of healthcare and data sharing networks. In light of this, it is imperative proper support be provided to public agencies to implement new technologies and remain part of healthcare and data sharing networks. CMS and ASTP can both be instrumental in supporting the inclusion and funding of IIS and other public health systems.

TD-18. Information blocking:

a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?

Public health programs are granted clear and definitive authority to access and utilize clinical health data appropriate to the goals and services of the program. Despite the clear authority granted to programs, we have seen resistance on the part of holders of healthcare data on engaging with public health to implement standards-based technology as well as non-responsiveness to queries. We encourage federal partners to emphasize the need, through regulation and standard operating procedures, if necessary, for data holders to engage with and

exchange with public health programs seeking appropriate data to support their activities.

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

As mentioned above, AIRA recommends that the function of responding to FHIR queries from authenticated PH programs through FHIR APIs become a requirement of the Public Health Exchange Purpose for TEFCA. As discussed, there are examples of Public Health Agencies who have modernized their systems to perform FHIR Queries only to be unable to receive responses, as this is not a requirement of TEFCA participants. Requiring response to a public health query will ultimately reduce provider burden through reduction in phone calls and other manual interventions.

In addition, we encourage ASTP/ONC and CMS to make it clear that health care organizations cannot charge public health for responding to a FHIR query. It is critical to avoid any disincentives that could keep either public health or private health care organizations from participating fully in TEFCA and/or appropriately sharing data with authorized trading partners.